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REPORT

PT Name : **MRS. SARWARI BANO**
Ref By : **DR. SMBT HOSPITAL**
Reg No : **HL8801827123 / RAJ47**
Barcode : **HED014591, HED014592**
INV : Aldosterone Renin Ratio

Age : 52 Year | **Sex :** Female
Registered on : 26-08-2026 01:11 PM
Received on : 26-08-2026 01:18 PM
Reported on : 27-08-2026 11:57 AM

SAMPLE COLLECTED AT :
SKY PATHOLOGY LAB

SAMPLE : Serum, EDTA_PLASMA

ALDOSTERONE RENIN RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Aldosterone Method: CLIA	16.8	ng/dl	- - -
REFERENCE Adults : 7–30 ng/dL Children : 5–80 ng/dL			
PLASMA RENIN ACTIVITY Method: RIA	2.5	ng/mL	- - -
REFERENCE **Reference Range :** POSTURE — REFERENCE RANGE OF PRA IN ng/mL/hr Upright — 1.90-6.00 Supine — 0.50-1.90			
Aldos/Renin Ratio Method: CALCULATED.	6.72	ng/mL/hr	0.00 - 30.0

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **CHAITALI PANDULE**
Ref By : **DR. SELF**
Reg No : **HL8000923253 / PUN162**
Barcode : **HE424712**
INV : Anti DNase Antibodies

Age : 24 Year | **Sex :** Female
Registered on : 20-08-2025 08:02 PM
Received on : 23-08-2025 05:59 PM
Reported on : 24-08-2025 04:43 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

ANTI DNASE ANTIBODIES

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ANTI DNASE ANTIBODIES Method: Nephelometry	140.2	U/mL	- - -

Reference Interval
<5 years: < or =250 U/mL
5-17 years < or =375 U/mL
> or =18 years: < or =300 U/mL

Clinical Information
A number of bacterial antigens have been identified in cultures of group A streptococci. These extracellular products are primarily enzymatic proteins and include streptolysin O, streptokinase, hyaluronidase, deoxyribonucleases (DNases A, B, C, and D), and nicotinamide adenine nucleotidase.

Infections by the group A streptococci are unique because they can be followed by serious nonpurulent complications of rheumatic fever and glomerulonephritis. Recent information suggests that rheumatic fever is associated with infection by certain rheumatogenic serotypes (M1, M3, M5, M6, M18, and M19), while glomerulonephritis follows infection by nephritogenic serotypes (M2, M12, M49, M57, M59, and M60).

Glomerulonephritis and rheumatic fever occur following the infection, after a period of latency following the infection, during which the patient is asymptomatic. The latency period for glomerulonephritis is approximately 10 days, and the latency period for rheumatic fever is 20 days.

Interpretation

Elevated values are consistent with an antecedent infection by group A streptococci. Although the antistreptolysin O (ASO) test is quite reliable, performing the anti-DNase is justified for 2 primary reasons. First, the ASO response is not universal. Elevated ASO titers are found in the sera of about 85% of individuals with rheumatic fever; ASO titers remain normal in about 15% of individuals with the disease. The same holds true for other streptococcal antibody tests: a significant portion of individuals with normal antibody titers for 1 test will have elevated antibody titers for another test. Thus, the percentage of false-negative results can be reduced by performing 2 or more antibody tests. Second, skin infections, in contrast to throat infections, are associated with a poor ASO response. Patients with acute glomerulonephritis following skin infection (post-impetigo) have an attenuated immune response to streptolysin O. For such patients, performance of an alternative streptococcal antibody test, such as this assay, is recommended.

~~End of report~~

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REPORT

PT Name	: **MS SAHITYA SETHURAMAN**	**Age :** 28 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** AKASH CLINICAL LABORATORY KALYAN
Ref By	: **DR. SUMIT SHRIVASTAVA**	**Registered on** : 30-08-2026 10:59 AM	
Reg No	: **HL8001830954 / MH640**	**Received on** : 30-08-2026 04:29 PM	
Barcode	: **HF324447**	**Reported on** : 07-09-2026 05:06 PM	**SAMPLE :** Blood culture bottle
INV	: BLOOD C/S BACTEC		

BLOOD C/S BACTEC

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

Preliminary Report - No bacterial growth was observed after 48 hours of aerobic incubation.
Secondary Report - No bacterial growth was observed after 72 hours of aerobic incubation.
Organism Isolated - Nil
Antibiogram - .
Sensitive to - Nil
Intermediate to - Nil
Resistant to - Nil
FINAL REPORT - No Bacterial Growth Was Observed After 1 week of Aerobic Incubation.
NOTE -

~~End of report~~

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REPORT

PT Name	: **SUNANDA CHAVAN**	**Age :** 51 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** ADVAIT CLINICAL LABORATORY
Ref By	: **DR. RAJ NAGARKAR**	**Registered on :** 26-08-2026 08:54 PM	
Reg No	: **HL3201827775 / MH004/MS102**	**Received on** : 29-08-2026 10:25 AM	
Barcode	: **HF486427**	**Reported on** : 29-08-2026 04:22 PM	**SAMPLE :** Serum
INV	: CHROMOGRANIN A; CGA		

CHROMOGRANIN A; CGA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
CHROMOGRANIN A; CGA	18.6	µg/L	0.0 - 100.0
Method: Enzyme Immunoassay			

INTERPRETATION

1. Human Chromogranin A is an acidic human protein. It is a tumor marker used to differentiate between endocrine and non endocrine tumors.
2. It is elevated in gastrinoma(100%), pheochromocytoma(89%), carcinoid tumors(80%), nonfunctioning tumors of the endocrine pancreas(69%) and medullary carcinoma of the thyroid(50%).
3. Healthy subjects may also show an elevated level (7%). Limitations: • Chromogranin A is not a diagnostic test for endocrine tumors. It can be elevated in patients with renal failure and in patients of atrophic chronic gastritis. • Proton pump inhibitors and H2-receptors antagonists should be discontinued for 10 days prior to the test for accurate result. • Patients treated by corticosteroids can show false increase in Chromogranin A.

~~End of report~~

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REPORT

PT Name : **KANCHAN KUMAR**
Ref By : **DR. GUFRAN ANSARI**
Reg No : **HL8401832480 / MH621**
Barcode : **HEI085207**
INV : DENGUE NS1 RAPID

Age : 39 Year | **Sex :** Male
Registered on : 31-08-2026 05:39 PM
Received on : 31-08-2026 06:00 PM
Reported on : 31-08-2026 07:00 PM

SAMPLE COLLECTED AT :
NEW LIFE CLINIC

SAMPLE : Serum

DENGUE NS1 RAPID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
DENGUE NS1 RAPID	POSITIVE	-	- - -

Method: -

Clinical significance :
Following dengue infection, the incubation period varies from 3 to 7 days, and while some infections remain asymptomatic, the majority of individuals will develop classic dengue fever. Symptomatic patients become acutely febrile and present with severe musculoskeletal pain, headache, retro-orbital pain, and a transient macular rash, most often observed in children. Fever defervescence signals disease resolution in most individuals. However, children and young adults remain at increased risk for progression to dengue hemorrhagic fever and dengue shock syndrome, particularly during repeat infection with a new DV serotype.

Detection of the DV nonstructural protein 1 (NS1) has emerged as an alternative biomarker to both serologic and molecular based techniques for diagnosis of acute DV infection. NS1 antigenemia is detectable within 24 hours and up to 9 days following symptoms onset. This overlaps with the DV viremic phase and NS1 is often detectable prior to IgM seroconversion. Concurrent evaluation for the NS1 antigen alongside testing for IgM- and IgG-class antibodies to DV (DENGM / Dengue Virus Antibody, IgG and IgM, Serum) provides optimal diagnostic potential for both early and late dengue disease.

~~End of report~~

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REPORT

PT Name	: **MR. RITIK WAGHELA**	**Age :** 26 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** GSDDPL
Ref By	: **DR. SELF**	**Registered on :** 31-08-2026 07:33 PM	
Reg No	: **HL3001832548 / HYMH/NALSPR/H7654**	**Received on** : 31-08-2026 08:10 PM	
Barcode	: **HEN111259**	**Reported on** : 02-09-2026 10:49 AM	**SAMPLE :** EDTA Blood
INV	: Dopamine		

DOPAMINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
DOPAMINE	55.6	pg/ml	15.0 - 100.0

Method: (Method: Enzyme immunoassay (EIA))

INTERPRETATION

Dopamine is a type of neurotransmitter and hormone. It plays a role in many important body functions, including movement, memory and pleasurable reward and motivation. High or low levels of dopamine are associated with several mental health and neurological diseases.

~~End of report~~

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REPORT

PT Name	: **MR LAXMAN JAGTAP**	**Age :** 72 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** AKASH CLINICAL LABORATORY KALYAN
Ref By	: **DR. RAJENDRA WANKHADE M.S.(ORTHO)**	**Registered on :** 21-08-2026 03:42 PM	
Reg No	: **HL3901821737 / MH640**	**Received on** : 21-08-2026 07:20 PM	
Barcode	: **HF324418**	**Reported on** : 22-08-2026 08:34 PM	
INV	: fluid For Z-N Stain		**SAMPLE :** Body Fluid

FLUID FOR Z-N STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN Method: .	FLUID	.	. - .
ORGANISM Method: Z-N STAIN	NOT SEEN	-	- - -

~~End of report~~

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REPORT

PT Name	: **LAKHANN SINGH**	**Age :** 58 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** SHRI RAM PATHOLOGY DHOLPUR
Ref By	: **DR. ANIMESH GUPTA GASTROENTEROLOGIST MBBS .MD.DM**	**Registered on :** 12-10-2025 11:37 AM	
Reg No	: **HL7001131770 / RJ011**	**Received on** : 12-10-2025 06:21 PM	
Barcode	: **Hdh120848**	**Reported on** : 12-10-2025 06:44 PM	
INV	: Hepatitis C Virus (HCV) IgM		**SAMPLE :** Serum

HEPATITIS C VIRUS (HCV) IGM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
HEPATITIS C VIRUS (HCV) IGM Method: ELISA	0.19	Index	0.00 - 0.90

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REPORT

PT Name : **MRS.SAKARI BEN**
Ref By : **DR. YASH BHANUSHALI**
Reg No : **HL3701648710 / GUJ016**
Barcode : **HEB185700**
INV : Liver Cytosolic Antigen Type 1 (LC-1) Antibody

Age : 71 Year | **Sex :** Female
Registered on : 06-05-2026 01:26 PM
Received on : 08-05-2026 07:01 PM
Reported on : 12-05-2026 03:50 PM

SAMPLE COLLECTED AT :
JAY PATH LABORATORY

SAMPLE : Serum

LIVER CYTOSOLIC ANTIGEN TYPE 1 (LC-1) ANTIBODY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIVER CYTOSOLIC ANTIGEN TYPE 1 (LC-1) Method: Qualitative Immunoblot	NEGATIVE	.	- - -

Interpretation

Anti-LC1 is an ELISA-based, automated, in-vitro test system for the quantitative determination of IgG antibodies against liver cytosolic protein type 1 (LC1) in human serum or plasma. It is used to support the differential diagnosis of liver diseases, particularly in cases of suspected type 2 autoimmune hepatitis.

The detection of autoantibodies plays a central role in the diagnosis of inflammatory liver diseases namely autoimmune hepatitis (AIH), primary biliary cirrhosis and sclerosing cholangitis in adults and children. Anti-smooth muscle antibodies (ASMA), anti-nuclear antibodies (ANA), and antibodies against the soluble liver antigen (anti-SLA) define AIH type 1. Liver-kidney microsome antibodies (anti-LKM-1) and liver cytosolic protein type 1 antibodies (anti-LC1) may be present in AIH type 2. Anti-LC1 may be the single antibody in up to one third of the patients with AIH-2, and AIH-2 patients with anti-LC1 antibodies have histologically more severe disease compared to those without anti-LC1 antibodies. Anti-LC1 was originally described in association with anti-LKM-1, or in isolation in patients with AIH-2. Anti-LC1 antibodies have also been found in 14 % anti-LKM-1 antibody positive patients suffering from chronic hepatitis C virus infection.

~~End of report~~

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REPORT

PT Name : **MR. SUNIL KAMBLE**
Ref By : **DR. AVINASH JADHAV**
Reg No : **HL9101831610 / MH516**
Barcode : **HEN144049**
INV : MALARIA ANTIGEN ELISA

Age : 40 Year | **Sex :** Male
Registered on : 30-08-2026 11:18 PM
Received on : 31-08-2026 03:17 AM
Reported on : 31-08-2026 07:06 AM

SAMPLE COLLECTED AT :
AJAY LAB

SAMPLE : Serum

MALARIA ANTIGEN ELISA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
MALARIA ANTIGEN Method: ELISA	0.21	Index	0.0 - 0.8 Negative result: ≤ 0.8 weekly positive: 0.8 - 1.0 Positive result: ≥ 1.0

Interpretation

A positive reaction corresponds to an Ag Index above or equal to 1.0 A negative reaction corresponds to an Ag Index below or equal to 0.8. A grey-zone of Ag Index between 0.8 and 1.0 has to be considered because some samples can produce higher backgrounds than others. This can mimic or mask low reactivity. Samples inside this “grey-zone” should be considered as doubtful and should be retested, preferably using a freshly drawn blood sample in order to assess a possible recent malaria infection..

~~End of report~~

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REPORT

PT Name : **SURJYA UCHOI**
Ref By : **DR. TATA HOSPITAL**
Reg No : **HL2901826963 / MH081**
Barcode : **HEN102473**
INV : Plasma Non Metanepherin Free

Age : 44 Year | **Sex :** Male
Registered on : 26-08-2026 12:06 PM
Received on : 26-08-2026 04:59 PM
Reported on : 28-08-2026 03:19 PM

SAMPLE COLLECTED AT :
DNA LAB
SAMPLE : EDTA_PLASMA

PLASMA NON METANEPHERIN FREE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Normetanephrines Free Method: (LC-MS/MS)	86.5	ng/l	20.10 - 135.40

Comment

Metanephrines and Normetanephrines collectively called Metanephrines are metabolites of Epinephrine and Norepinephrine respectively. They are elevated in catecholamine secreting tumors like Pheochromocytomas and in tumors derived from neural crest cells like Paraganglioma and Neuroblastoma. Measurement of plasma free metanephrines appears to be the best test for excluding pheochromocytoma as the test's sensitivity is approximately 96-99 %. It offers a specificity between 80-100 % and therefore relatively high rate of false positive results may be found. Several preanalytical factors may affect the test results such as exercise, posture, food, stress, hypoglycemia and certain medications etc. Due to the low prevalence of pheochromocytomas and related tumors, it is recommended to confirm elevated plasma free metanephrines with a second line test. The recommended second -line test is measurement of fractionated 24-hour urinary metanephrines. Tumoural production of 3-methoxytyramine is associated with presence of an underlying SDHB mutation and may be a biomarker of malignancy.

~~End of report~~

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REPORT

PT Name	: **CHANDRAVATI DEVI**	**Age :** 58 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SATRUPA DEB**	**Registered on :** 30-08-2026 01:10 PM	RX PATHOLOGY
Reg No	: **HL5401831158 / RAJ40**	**Received on** : 30-08-2026 04:20 PM	
Barcode	: **HED058411**	**Reported on** : 30-08-2026 08:37 PM	
INV	: Sodium,Potassium		**SAMPLE :** Serum

SODIUM,POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
SODIUM Method: Electrode	**132.7**	mEq/L	133 - 146
POTASSIUM Method: Electrode	4.4	mEq/L	3.8 - 5.4

Interpreting test results

Your electrolyte panel results will show the concentrations of sodium, potassium, chloride, and bicarbonate in your blood. The amounts are measured and reported in millimoles per liter (mmol/L) or milliequivalents per liter (mEq/L). If other tests were conducted at the same time, you might see other measurements on your test report.

The test report typically includes reference ranges for each electrolyte. These values are what the laboratory considers to be the expected values if you are healthy. Reference ranges may vary slightly from laboratory to laboratory. Because of variations in values and interpretation, it is important to discuss your results with your physician before drawing any conclusions about your electrolyte levels.

Many different health conditions can cause abnormal levels of electrolytes. Interpreting your test result involves considering which electrolyte levels are abnormal, whether they are high or low, your symptoms, and your health history. Some health issues that can affect electrolyte levels and the anion gap include

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REPORT

PT Name	: **MRS. TARANNUM KHAN**	**Age :** 30 Y | **Sex :** Female	**SAMPLE COLLECTED AT :** LIFE CARE PATHOLOGY
Ref By	: **DR. LIFE CARE PATHOLOGY**	**Registered on :** 26-08-2026 09:23 PM	
Reg No	: **HL2101827799 / MH713**	**Received on :** 27-08-2026 04:53 AM	
Barcode	: **HEN143902**	**Reported on :** 27-08-2026 07:46 AM	
INV	: TORCH-IgG-5		**SAMPLE :** Serum

TORCH-IGG-5

TEST DESCRIPTION	RESULT	UNITS	RANGES
Toxoplasma gondii IgG Method: CLIA	0.65	IU/mL	0.0-3.0
Rubella (German Measles) IgG Method: CLIA	1.85	IU/mL	0.0-10.0
Cytomegalo Virus (CMV) IgG Method: CLIA	**175.0**	IU/mL	0.0-30.0
HERPES SIMPLEX VIRUS (HSV-1) -IgG Method: CLIA	**35.6**	IU/mL	0.0-30.0
HERPES SIMPLEX VIRUS (HSV-2) -IgG Method: CLIA	17.85	IU/mL	0.0-30.0

INTERPRETATION

***Toxoplasma gondii*:**
A positive *Toxoplasma* IgG result is indicative of current or past infection with *T gondii*. A single positive *Toxoplasma* IgG result should not be used to diagnose recent infection.Equivocal *Toxoplasma* IgG results may be due to very low levels of circulating IgG during the acute stage of infection. A second specimen should be submitted for testing if clinically indicated.Individuals with negative *Toxoplasma* IgG results are presumed to not have had previous exposure to *T gondii* . However, negative results may be seen in cases of remote exposure with subsequent loss of detectable antibody. Seroconversion from negative to positive IgG is indicative of *T gondii* infection subsequent to the first negative specimen.

Rubella:
Positive: The presence of detectable IgG-class antibodies to rubella indicates prior exposure through infection or immunization. Individuals testing positive for IgG-class antibodies to rubella are considered immune.Equivocal: Submit an additional specimen for testing in 10 to 14 days to demonstrate IgG seroconversion if recently vaccinated or if otherwise clinically indicated.Negative: The absence of detectable IgG-class antibodies to rubella suggests no prior exposure to this virus or the lack of a specific immune response to immunization.

Cytomegalovirus:
Positive cytomegalovirus (CMV) IgG results indicate past or recent CMV infection. These individuals may transmit CMV to susceptible individuals through blood and tissue products.Equivocal CMV IgG results may occur during acute infection or may be due to nonspecific binding reactions. Submit an additional specimen for testing if clinically indicated.Individuals with negative CMV IgG results are presumed to not have had prior exposure or infection with CMV and are therefore considered susceptible to primary infection.

Herpes Simplex Virus:
The presence of IgG-class antibodies to herpes simplex virus (HSV) types 1 or 2 indicates previous exposure and does not necessarily indicate that HSV is the causative agent of an acute illness.

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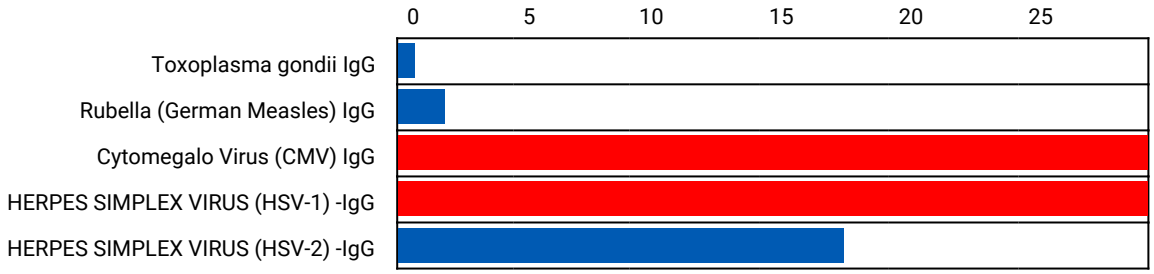
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REPORT

PT Name	: **MRS. TARANNUM KHAN**	**Age :** 30 Y | **Sex :** Female	**SAMPLE COLLECTED AT :** LIFE CARE PATHOLOGY
Ref By	: **DR. LIFE CARE PATHOLOGY**	**Registered on :** 26-08-2026 09:23 PM	
Reg No	: **HL2101827799 / MH713**	**Received on** : 27-08-2026 04:53 AM	
Barcode	: **HEN143902**	**Reported on** : 27-08-2026 07:46 AM	
INV	: TORCH-IgG-5		**SAMPLE :** Serum

MOLECULAR BIOLOGY



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REPORT

PT Name : **MRS. LAXMI**
Ref By : **DR. SELF**
Reg No : **HL9901829120 / MH525**
Barcode : **hen118483**
INV : URINE Glucose (Fasting),

Age : 44 Year | **Sex :** Female
Registered on : 28-08-2026 12:13 PM
Received on : 28-08-2026 02:08 PM
Reported on : 28-08-2026 08:56 PM

SAMPLE COLLECTED AT :
SERUM DIAGNOSTIC

SAMPLE : Urine

URINE GLUCOSE (FASTING),

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINE GLUCOSE (FASTING) Method: Reagent strip	92.68	mg/dL	70.0 - 140.0 141.0 - 200.0 : Prediabetic >200.0 : Diabetic

Interpretation :

When we eat food, the pancreas (an organ that sits between the stomach and the spine) goes to work, releasing enzymes that help to break down food and hormones that help the body handle the influx of glucose. One of these hormones is insulin, and it plays a key role in managing glucose levels in the blood.

And here is where things can go wrong. If the pancreas doesn't make enough insulin — or stops making it altogether, in the case of type 1 diabetes — glucose levels in the blood can rise too high. Another scenario is that the pancreas makes enough insulin but the cells have trouble using it properly, causing blood glucose levels to rise. This is called insulin resistance and is the hallmark of type 2 diabetes.

In the short term, high blood glucose levels can make you feel downright bad. Thirst, frequent trips to the bathroom, fatigue, and weight loss are all symptoms of high blood glucose (hyperglycemia). If not treated, more serious issues can occur, such as diabetic ketoacidosis (DKA). Chronic high blood glucose levels can lead to complications such as heart, kidney and eye disease, as well as nerve damage. So, it's all about the blood glucose.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MRS. SADIYA BANO**	**Age :** 36 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** NEW LIFE CARE
Ref By	: **DR. DR.SADIQA .M.A.**	**Registered on :** 27-08-2026 02:44 PM	
Reg No	: **HL1501828390 / MH423**	**Received on** : 27-08-2026 05:40 PM	
Barcode	: **HEN158947**	**Reported on** : 27-08-2026 09:25 PM	
INV	: URINE Glucose (PP)		**SAMPLE :** Urine

URINE GLUCOSE (PP)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Glucose, Post Prandial (PP) Method: GOD-POD	116.8	mg/dl	70.0 - 140.0 141.0 - 200.0 : Prediabetic >200.0 : Diabetic

Interpretation :

When we eat food, the pancreas (an organ that sits between the stomach and the spine) goes to work, releasing enzymes that help to break down food and hormones that help the body handle the influx of glucose. One of these hormones is insulin, and it plays a key role in managing glucose levels in the blood.

And here is where things can go wrong. If the pancreas doesn't make enough insulin — or stops making it altogether, in the case of type 1 diabetes — glucose levels in the blood can rise too high. Another scenario is that the pancreas makes enough insulin but the cells have trouble using it properly, causing blood glucose levels to rise. This is called insulin resistance and is the hallmark of type 2 diabetes.

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~~End of report~~

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REPORT

PT Name	: **HEENA SHAIKH**	**Age :** 34 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. AASHIRWAD HOSPITAL**	**Registered on** : 31-08-2026 11:37 PM	
Reg No	: **HL6101832572 / RAJ41**	**Received on** : 01-09-2026 04:02 AM	
Barcode	: **HEN176429**	**Reported on** : 01-09-2026 02:27 PM	
INV	: URINE PROTEIN & CRETININE RANDOM		**SAMPLE :** Urine

URINE PROTEIN & CRETININE RANDOM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Urinary Protein, Total Method: Spectrophotometry	**870.4**	mg/dL	0.0 - 15.0
Urinary Creatinine Method: Spectrophotometry	87.9	mg/dL	28.0 - 217.0

~~End of report~~

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REPORT

PT Name	: **MR NITYANAND SINGH**	**Age :** 60 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. ANUJ BHASIN**	**Registered on** : 31-08-2026 11:37 AM	
Reg No	: **HL6101831854 / MH755**	**Received on** : 31-08-2026 05:06 PM	
Barcode	: **HED093166**	**Reported on** : 31-08-2026 06:29 PM	
INV	: Vitamin D3		**SAMPLE :** Serum

VITAMIN D3

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Vitamin D3 Method: CLIA	39.16	ng/mL	. - . Deficiency: <20 Insufficiency: 20 - 30 Sufficiency: 30 - 100 Toxicity: > 100

Interpretation

1. The 25-hydroxyvitamin D (Vitamin D3) is the major form found in the blood and is the relatively inactive precursor to the active hormone, 1,25-dihydroxyvitamin D. Because of its long half-life and higher concentration, 25-hydroxyvitamin D is commonly measured to assess and monitor vitamin D status in individuals. A low blood level of 25-hydroxyvitamin D may mean that a person is not getting enough exposure to sunlight or enough dietary vitamin D to meet his or her body's demand or that there is a problem with its absorption from the intestines.

2. Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol (Vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(Vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.

3. Testing for 25(OH) vitamin D (Vitamin D3) is recommended as it is the best indicator of vitamin D nutritional status.

~~End of report~~

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